PharmEasy Labs

Name : Chameli Debnath(33Y/F)

Date : 6 Mar, 2026

Test Asked : Ekcg142406hc

Report Status : Complete Report

REPORT VERIFIED
VERIFIED
BY DOCTOR

6 STEP quality control to ensure 100% report accuracy

- Qualified and trained technicians
- Temperature-controlled containers to store samples
- Strict quality checks on samples before processing
- Regular monitoring of lab analyzers by experts
- Assured machine inspection on a daily basis
- Verified reports by qualified pathologists

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DOCON TECHNOLOGIES PRIVATE LIMITED, Office No.208, 209, 210 Second floor, A wing, 'Dattani Plaza', Near East West Industrial Estate, Safed Pool, Saki Naka, Andheri (East), Mumbai - 400072

Patient Name	: **CHAMELI DEBNATH(33Y/F)**	Tests Done	: **EKCG142406HC**
Referred By	: **SELF**		
Home Collection :	**K29/104 SHAPOORJI PALLONJI SUKHOBRISHTI COMPLEX ROAD ACTION AREA III NEWTOWN NEW TOWN WEST BENGAL INDIA 700135**		

Report Availability Summary

Note: Please refer to the table below for status of your tests.

13 Ready | **0** Ready with Cancellation | **0** Processing | **0** Cancelled in Lab

TEST DETAILS	REPORT STATUS
EKCG142406HC	Ready
PERIPHERAL BLOOD SMEAR (PBS)	Ready
ERYTHROCYTE SEDIMENTATION RATE (ESR)	Ready
HEMOGRAM - 6 PART (DIFF)	Ready
CREATININE - SERUM	Ready
HbA1c	Ready
VITAMIN B-12	Ready
URIC ACID	Ready
LIPID PROFILE	Ready
T3-T4-USTSH	Ready
FASTING BLOOD SUGAR(GLUCOSE)	Ready
BLOOD UREA NITROGEN (BUN)	Ready
LIVER FUNCTION TESTS	Ready
25-OH VITAMIN D (TOTAL)	Ready

Patient Name	: CHAMELI DEBNATH(33Y/F)	Tests Done	: EKCG142406HC
Referred By	: SELF		
Home Collection :	K29/104 SHAPOORJI PALLONJI SUKHOBRISHTI COMPLEX ROAD ACTION AREA III NEWTOWN NEW TOWN WEST BENGAL INDIA 700135		

Tests Outside Reference Range

Note: Please refer to the table below for tests outside reference range.

Test Name	Observed Value	Units	Bio. Ref. Interval.
COMPLETE HEMOGRAM			
HEMOGLOBIN	**11.6**	g/dL	12.0-15.0
MEAN CORP.HEMO.CONC(MCHC)	**29.3**	g/dL	31.5-34.5
MEAN CORPUSCULAR HEMOGLOBIN(MCH)	**26**	pq	27.0-32.0
MEAN PLATELET VOLUME(MPV)	**13.4**	fL	6.5-12
MONOCYTES - ABSOLUTE COUNT	**0.17**	X 10^3 / µL	0.2 - 1.0
PLATELET DISTRIBUTION WIDTH(PDW)	**18.5**	fL	9.6-15.2
PLATELET TO LARGE CELL RATIO(PLCR)	**52.8**	%	19.7-42.4
RED CELL DISTRIBUTION WIDTH (RDW-CV)	**15.6**	%	11.6-14.0
RED CELL DISTRIBUTION WIDTH - SD(RDW-SD)	**50.4**	fL	39.0-46.0
LIPID			
HDL / LDL RATIO	**0.28**	Ratio	> 0.40
HDL CHOLESTEROL - DIRECT	**33**	mg/dL	40-60
LDL / HDL RATIO	**3.5**	Ratio	1.5-3.5
LDL CHOLESTEROL - DIRECT	**117**	mg/dL	< 100
TC/ HDL CHOLESTEROL RATIO	**5.1**	Ratio	3 - 5
LIVER			
GAMMA GLUTAMYL TRANSFERASE (GGT)	**40.51**	U/L	< 38
OTHER COUNTS			
ERYTHROCYTE SEDIMENTATION RATE (ESR)	**50**	mm / hr	<20
RENAL			
BLOOD UREA NITROGEN (BUN)	**6.3**	mg/dL	7.94 - 20.07
CREATININE - SERUM	**0.48**	mg/dL	0.55-1.02
UREA (CALCULATED)	**13.48**	mg/dL	Adult : 17-43
THYROID			
TSH - ULTRASENSITIVE	**5.46**	µIU/mL	0.54-5.30
VITAMIN			
25-OH VITAMIN D (TOTAL)	**28.9**	ng/mL	30-100

Disclaimer: The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.

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Patient Name	: CHAMELI DEBNATH(33Y/F)		Sample Collected on (SCT)	: 06 Mar 2026 06:07
Referred By	: SELF		Sample Received on (SRT)	: 06 Mar 2026 15:58
Home Collection	: K29/104 SHAPOORJI PALLONJI SUKHOBRISHTI COMPLEX ROAD ACTION AREA III NEWTOWN NEW TOWN WEST BENGAL INDIA 700135		Report Released on (RRT)	: 06 Mar 2026 20:10
			Sample Type | Barcode	: EDTA Whole Blood | EQ065722

TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c	H.P.L.C	4.8	%

Bio. Ref. Interval. :

As per ADA Guidelines	
Below 5.7%	: Normal
5.7% - 6.4%	: Prediabetic
>=6.5%	: Diabetic

Guidance For Known Diabetics	
Below 6.5%	: Good Control
6.5% - 7%	: Fair Control
7.0% - 8%	: Unsatisfactory Control
>8%	: Poor Control

Method : Fully Automated H.P.L.C method

TEST NAME	TECHNOLOGY	VALUE	UNITS
AVERAGE BLOOD GLUCOSE (ABG)	CALCULATED	91	mg/dL

Bio. Ref. Interval. :

90 - 120 mg/dl	: Good Control
121 - 150 mg/dl	: Fair Control
151 - 180 mg/dl	: Unsatisfactory Control
> 180 mg/dl	: Poor Control

Method : Derived from HBA1c values

Please correlate with clinical conditions.

Tests Done : EKCG142406HC

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

Mohini Gupta

Dr.Mohini Gupta
MD(Path)

Scan QR to verify(valid for 30 days from release time)

Processed At :
106, Shreegopal, Jessore Rd,
Madhyamgram Kolkata-700129

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TEST NAME	TECHNOLOGY	VALUE	UNITS
ERYTHROCYTE SEDIMENTATION RATE (ESR)	**MODIFIED WESTERGREN**	**50**	**mm / hr**

Bio. Ref. Interval. :-

<50 yr : Male: 0-15 mm/hr Female: 0-20 mm/hr
>50 yr : Male: 0-20 mm/hr Female: 0-30 mm/hr
Children: <=10 mm/hr

Clinical Significance:
- An erythrocyte sedimentation rate (ESR) is a blood test that can rise if you have inflammation in your body.Its also used as a marker to monitor prognosis of an existing inflammatory/infective condition.
- Inflammation is your immune systems response to injury, infection, and many types of conditions, including immune system disorders, certain cancers and blood disorders.
- A high ESR test result may be from a condition that causes inflammation, such as: Arteritis, Arthritis, Systemic vasculitis, Polymyalgia rheumatica, Inflammatory bowel disease, Kidney disease, Infections like Tuberculosis etc, Rheumatoid arthritis and other autoimmune diseases, Heart disease, Certain cancers and many other Conditions.
- A low ESR test result may be caused by conditions such as: A blood disorder, such as: Polycythemia, Sickle cell disease (SCD), Leukocytosis, Heart failure, Certain kidney and liver problems etc.
- Certain physiological conditions also affect ESR results, these include : Pregnancy, menstrual cycle, ageing, obesity, drinking alcohol regularly, and exercise, Certain medicines and supplements also can affect ESR results.
- Hence Its always suggested to interpret ESR results in conjunction with Clinical History and other findings.

References :
https://medlineplus.gov/lab-tests/erythrocyte-sedimentation-rate-esr/

Please correlate with clinical conditions.
Method:- MODIFIED WESTERGREN

Tests Done : EKCG142406HC

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL accredited, NABL certificate no: MC-4948

Mohini Gupta

Dr.Mohini Gupta
MD(Path)

Dr Arpita MD (Path)

Processed At :
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Madhyamgram Kolkata-700129

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TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	**SLS-Hemoglobin Method**	**11.6**	**g/dL**	**12.0-15.0**
Hematocrit (PCV)	CPH Detection	39.6	%	36.0-46.0
Total RBC	HF & EI	4.46	X 10^6/µL	3.8-4.8
Mean Corpuscular Volume (MCV)	Calculated	88.8	fL	83.0-101.0
Mean Corpuscular Hemoglobin (MCH)	**Calculated**	**26**	**pq**	**27.0-32.0**
Mean Corp.Hemo. Conc (MCHC)	**Calculated**	**29.3**	**g/dL**	**31.5-34.5**
Red Cell Distribution Width - SD (RDW-SD)	**Calculated**	**50.4**	**fL**	**39.0-46.0**
Red Cell Distribution Width (RDW - CV)	**Calculated**	**15.6**	**%**	**11.6-14.0**
RED CELL DISTRIBUTION WIDTH INDEX (RDWI)	Calculated	310.6	-	*Refer Note below
MENTZER INDEX	Calculated	19.9	-	*Refer Note below
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	7.46	X 10³ / µL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	73.9	%	40-80
Lymphocytes Percentage	Flow Cytometry	22	%	20-40
Monocytes Percentage	Flow Cytometry	2.3	%	2-10
Eosinophils Percentage	Flow Cytometry	1.2	%	1-6
Basophils Percentage	Flow Cytometry	0.3	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.3	%	0.0-0.4
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	5.51	X 10³ / µL	2.0-7.0
Lymphocytes - Absolute Count	Calculated	1.64	X 10³ / µL	1.0-3.0
Monocytes - Absolute Count	**Calculated**	**0.17**	**X 10³ / µL**	**0.2 - 1.0**
Basophils - Absolute Count	Calculated	0.02	X 10³ / µL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.09	X 10³ / µL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.02	X 10³ / µL	0.0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10³ / µL	0.0-0.5
PLATELET COUNT	HF & EI	195	X 10³ / µL	150-410
Mean Platelet Volume (MPV)	**Calculated**	**13.4**	**fL**	**6.5-12**
Platelet Distribution Width (PDW)	**Calculated**	**18.5**	**fL**	**9.6-15.2**
Platelet to Large Cell Ratio (PLCR)	**Calculated**	**52.8**	**%**	**19.7-42.4**
Plateletcrit (PCT)	Calculated	0.26	%	0.19-0.39

Remarks : Alert!!! RBCs:Mild anisopoikilocytosis. Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

*Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.
Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)
(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height)

Tests Done : EKCG142406HC

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

Mohini Gupta

Dr.Mohini Gupta
MD(Path)

Dr Arpita MD (Path)

Processed At :
106, Shreegopal, Jessore Rd,
Madhyamgram Kolkata-700129

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		Sample Type | Barcode	: EDTA Whole Blood | EQ065722

TEST NAME	METHOD
PERIPHERAL BLOOD SMEAR (PBS)	MICROSCOPY

RBCs : Mild anisopoikilocytosis. Predominantly normocytic normochromic with ovalocytes. Total population is normal in smear.

WBCs : Total count is normal with normal morphology in smear.

PLATELET : Appear adequate in smear with normal morphology.

CLINICAL REMARKS : Clinical correlation & follow up.

TECHNOLOGY : MICROSCOPY

Please correlate with clinical conditions

Tests Done : EKCG142406HC

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL accredited, NABL certificate no: MC-4948

Dr.Mohini Gupta
MD(Path)

Dr Arpita MD (Path)

PharmEasy Labs
7022 000 900

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Patient Name	: CHAMELI DEBNATH(33Y/F)	Sample Collected on (SCT)	: 06 Mar 2026 06:07
Referred By	: SELF	Sample Received on (SRT)	: 06 Mar 2026 15:56
Home Collection	: K29/104 SHAPOORJI PALLONJI SUKHOBRISHTI COMPLEX ROAD ACTION AREA III NEWTOWN NEW TOWN WEST BENGAL INDIA 700135	Report Released on (RRT)	: 06 Mar 2026 16:51
		Sample Type | Barcode	: FLUORIDE PLASMA | EQ065724

TEST NAME	TECHNOLOGY	VALUE	UNITS
FASTING BLOOD SUGAR(GLUCOSE)	PHOTOMETRY	97	mg/dL

Bio. Ref. Interval. :-

As per ADA Guideline: Fasting Plasma Glucose (FPG)	
Normal	70 to 100 mg/dl
Prediabetes	100 mg/dl to 125 mg/dl
Diabetes	126 mg/dl or higher

Note :
The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed , icteric or lipemic. The concentration of Glucose in a given specimen may vary due to differences in assay methods, calibration and reagent specificity. For diagnostic purposes results should always be assessed in conjunction with patients medical history, clinical findings and other findings.

Please correlate with clinical conditions.
Method:- GOD-POD METHOD

Tests Done : EKCG142406HC

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL accredited, NABL certificate no: MC-4948

Mohini Gupta

Dr.Mohini Gupta
MD(Path)

Processed At :
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		Sample Type | Barcode	: SERUM | EQ065723

TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL)	**E.C.L.I.A**	**28.9**	**ng/mL**

Bio. Ref. Interval. :-

Deficiency : <=20 ng/ml || Insufficiency : 21-29 ng/ml
Sufficiency : >= 30 ng/ml || Toxicity : >100 ng/ml

Clinical Significance:
Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health.
Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome.
Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):9.20%, Inter assay (%CV):8.50%
Kit Validation Reference : Holick M. Vtamin D the underappreciated D-Lightful hormone that is important for Skeletal and cellular health Curr Opin Endocrinol Diabetes 2002:9(1)87-98.

Please correlate with clinical conditions.
Method:- Fully Automated Electrochemiluminescence Compititive Immunoassay

Tests Done : EKCG142406HC

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL accredited, NABL certificate no: MC-4948

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		Sample Type | Barcode	:	SERUM | EQ065723

TEST NAME	TECHNOLOGY	VALUE	UNITS
VITAMIN B-12	E.C.L.I.A	256	pg/mL

Bio. Ref. Interval. :-

Normal: 197-771 pg/ml

Clinical significance :
Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):2.6%, Inter assay (%CV):2.3 %

Kit Validation Reference : Thomas L.Clinical laborator Diagnostics : Use and Assessment of Clinical laboratory Results 1st Edition,TH Books-Verl-Ges,1998:424-431

Please correlate with clinical conditions.

Method:- Fully Automated Electrochemiluminescence Compititive Immunoassay

Tests Done : EKCG142406HC

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		Sample Type | Barcode	:	SERUM | EQ065723

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
TOTAL CHOLESTEROL	PHOTOMETRY	168	mg/dL	< 200
HDL CHOLESTEROL - DIRECT	**PHOTOMETRY**	**33**	**mg/dL**	**40-60**
LDL CHOLESTEROL - DIRECT	**PHOTOMETRY**	**117**	**mg/dL**	**< 100**
TRIGLYCERIDES	PHOTOMETRY	77	mg/dL	< 150
TC/ HDL CHOLESTEROL RATIO	**CALCULATED**	**5.1**	**Ratio**	**3 - 5**
TRIG / HDL RATIO	CALCULATED	2.33	Ratio	< 3.12
LDL / HDL RATIO	**CALCULATED**	**3.5**	**Ratio**	**1.5-3.5**
HDL / LDL RATIO	**CALCULATED**	**0.28**	**Ratio**	**> 0.40**
NON-HDL CHOLESTEROL	CALCULATED	134.81	mg/dL	< 160
VLDL CHOLESTEROL	CALCULATED	15.47	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDL - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

Tests Done : EKCG142406HC

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL accredited, NABL certificate no: MC-4948

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MD(Path)

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		Sample Type | Barcode	:	SERUM | EQ065723

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
ALKALINE PHOSPHATASE	PHOTOMETRY	121.78	U/L	45-129
BILIRUBIN - TOTAL	PHOTOMETRY	0.7	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.16	mg/dL	0 - 0.20
BILIRUBIN (INDIRECT)	CALCULATED	0.54	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	**PHOTOMETRY**	**40.51**	**U/L**	**< 38**
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	27.41	U/L	< 31
ALANINE TRANSAMINASE (SGPT)	PHOTOMETRY	25.8	U/L	< 34
SGOT / SGPT RATIO	CALCULATED	1.06	Ratio	< 2
PROTEIN - TOTAL	PHOTOMETRY	6.68	gm/dL	5.7-8.2
ALBUMIN - SERUM	PHOTOMETRY	3.86	gm/dL	3.2-4.8
SERUM GLOBULIN	CALCULATED	2.82	gm/dL	2.5-3.4
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.37	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Diazonium salt DPD method
BILD - Diazonium salt DPD method
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
OT/PT - Derived from SGOT and SGPT values.
PROT - Biuret Method
SALB - Albumin Bcg¹method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

Tests Done : EKCG142406HC

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

Mohini Gupta

Dr.Mohini Gupta
MD(Path)

Processed At :
106, Shreegopal, Jessore Rd,
Madhyamgram Kolkata-700129

PharmEasy Labs

PharmEasy Labs
7022 000 900

DOCON TECHNOLOGIES PRIVATE LIMITED, Office No.208, 209, 210 Second floor, A wing, 'Dattani Plaza', Near East West Industrial Estate, Safed Pool, Saki Naka, Andheri (East), Mumbai - 400072

Patient Name	: CHAMELI DEBNATH(33Y/F)	Sample Collected on (SCT)	:	06 Mar 2026 06:07
Referred By	: SELF	Sample Received on (SRT)	:	06 Mar 2026 16:00
Home Collection	: K29/104 SHAPOORJI PALLONJI SUKHOBRISHTI COMPLEX ROAD ACTION AREA III NEWTOWN NEW TOWN WEST BENGAL INDIA 700135	Report Released on (RRT)	:	06 Mar 2026 18:28
		Sample Type | Barcode	:	SERUM | EQ065723

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
BLOOD UREA NITROGEN (BUN)	**PHOTOMETRY**	**6.3**	**mg/dL**	**7.94 - 20.07**
UREA / SR.CREATININE RATIO	CALCULATED	28.09	Ratio	< 52
UREA (CALCULATED)	**CALCULATED**	**13.48**	**mg/dL**	**Adult : 17-43**
BUN / Sr.CREATININE RATIO	CALCULATED	13.13	Ratio	9:1-23:1
URIC ACID	PHOTOMETRY	4	mg/dL	3.2 - 6.1

Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.
UR/CR - Derived from UREA and Sr.Creatinine values.
UREAC - Derived from BUN Value.
B/CR - Derived from serum Bun and Creatinine values
URIC - Uricase / Peroxidase Method

Tests Done : EKCG142406HC

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL accredited, NABL certificate no: MC-4948

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			Sample Type | Barcode	: SERUM | EQ065723

TEST NAME	TECHNOLOGY	VALUE	UNITS
CREATININE - SERUM	**PHOTOMETRY**	**0.48**	**mg/dL**

Bio. Ref. Interval. :-

Male : 0.72 -1.18 mg/dL
Female: 0.55 - 1.02 mg/dL

Clinical Significance :
The significance of a single creatinine value must be interpreted in light of the patients muscle mass. A patient with a greater muscle mass will have a higher creatinine concentration. The trend of serum creatinine concentrations over time is more important than absolute creatinine concentration. Serum creatinine concentrations may increase when an ACE inhibitor (ACEI) is taken. The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed , icteric or lipemic.

Please correlate with clinical conditions.
Method:- Creatinine Enzymatic Method

Tests Done : EKCG142406HC

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL accredited, NABL certificate no: MC-4948

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	E.C.L.I.A	148	ng/dL	80-200
TOTAL THYROXINE (T4)	E.C.L.I.A	10.6	µg/dL	4.8-12.7
TSH - ULTRASENSITIVE	**E.C.L.I.A**	**5.46**	**µIU/mL**	**0.54-5.30**

Comments : ***

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3,T4 - Fully Automated Electrochemiluminescence Compititive Immunoassay
USTSH - Fully Automated Electrochemiluminescence Sandwich Immunoassay

Pregnancy reference ranges for TSH/USTSH :

Trimester	T3 (ng/dl)	T4 (µg/dl)	TSH/USTSH (µIU/ml)
1st	83.9-196.6	4.4-11.5	0.1-2.5
2nd	86.1-217.4	4.9-12.2	0.2-3.0
3rd	79.9-186	5.1-13.2	0.3-3.5

References :

1. Carol Devilia, C I Parhon. First Trimester Pregnancy ranges for Serum TSH and Thyroid Tumor reclassified as Benign. Acta Endocrinol. 2016; 12(2) : 242 - 243
2. Kulhari K, Negi R, Kalra DK et al. Establishing Trimester specific Reference ranges for thyroid hormones in Indian women with normal pregnancy : New light through old window. Indian Journal of Contemporary medical research. 2019; 6(4)

Disclaimer : Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

Tests Done : EKCG142406HC

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Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd-NABL accredited, NABL certificate no: MC-4948

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			Sample Type | Barcode	: SERUM | EQ065723

TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR)	CALCULATED	128	mL/min/1.73 m2

Bio. Ref. Interval. :-

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease
<15 : Kidney Failure

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.
Method:- 2021 CKD EPI Creatinine Equation

~~ End of report ~~

CONDITIONS OF REPORTING

- The reported results are for information and interpretation of the referring doctor only.
- It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- This report is not valid for medico-legal purpose.
- Docon Technologies Private Limited,Thyrocare Technologies Limited and its employees/representatives do not assume any liability,responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report.

EXPLANATIONS

- **Name -** The name is as declared by the client and recorded by the personnel who collected the specimen.
- **Ref.By -** The name of the doctor who has recommended testing as declared by the client.
- **Labcode -** This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- **Barcode -** This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- **SCT -** Specimen Collection Time - The time when specimen was collected as declared by the client.
- **SRT -** Specimen Receiving Time - This time when the specimen reached our laboratory.
- **RRT -** Report Releasing Time - The time when our pathologist has released the values for Reporting.
- **Reference Range -** Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- Values out of reference range requires reconfirmation before starting any medical treatment.
- Retesting is needed if you suspect any quality shortcomings.
- For suggestions, complaints or feedback, write to us at grievance-office@docon.co.in or call us on 7022000900.